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Influenza: Protecting Individuals and Communities through Prevention, Early Recognition and Surveillance - I

This is the first article of two in a series on "Influenza: Protecting Individuals and Communities through Prevention, Early Recognition and Surveillance"

Seasonal influenza is an acute, contagious respiratory infection that causes substantial morbidity and mortality worldwide. Although most affected individuals recover without complications, influenza can cause severe illness, hospitalization and death, particularly among young children, older adults, pregnant women and individuals with underlying chronic medical conditions. In Sri Lanka, influenza circulates throughout the year, with periods of increased activity generally observed from May to July and November to January. Prevention requires a combination of public awareness, respiratory hygiene, appropriate healthcare-seeking behaviours and effective surveillance. The Epidemiology Unit of the Ministry of Health coordinates national influenza surveillance through influenza-like illness (ILI) and severe acute respiratory infection (SARI) sentinel surveillance, supported by laboratory testing through the National Influenza Centre at the Medical Research Institute. This article highlights the importance of recognizing influenza symptoms, preventing transmission, identifying individuals at increased risk of complications and strengthening surveillance to guide timely public health action.

Introduction

Influenza, commonly known as the flu, is an acute respiratory infection caused mainly by influenza A and B viruses. It remains an important public health concern because of its potential to cause seasonal epidemics, severe respiratory illness and excess demand for healthcare services. The World Health Organization (WHO) estimates that seasonal influenza causes approximately one billion cases annually worldwide, including three to five million cases of severe illness and 290,000–650,000 respiratory deaths [1].

Influenza should not be regarded simply as a common cold. While many people recover within a week, the infection can progress to pneumonia, worsen existing chronic diseases and result in hospitalisation or death. Protecting vulnerable populations and reducing community transmission are therefore essential components of influenza prevention and control.

Influenza in Sri Lanka

As a tropical country, Sri Lanka experiences influenza circulation throughout the year, with recognized periods of increased activity generally occurring from May to July and November to January [2]. The timing and intensity of these peaks may vary between years depending on circulating viruses and other epidemiological factors.

The Epidemiology Unit coordinates national human influenza surveillance through two complementary components: influenza-like illness (ILI) surveillance among outpatients and severe acute respiratory infection (SARI) surveillance among hospitalized patients. These systems provide information on respiratory disease trends and the severity of illness. Virological surveillance, coordinated with the National Influenza Centre at the Medical Research Institute, supports the identification and monitoring of circulating influenza viruses.

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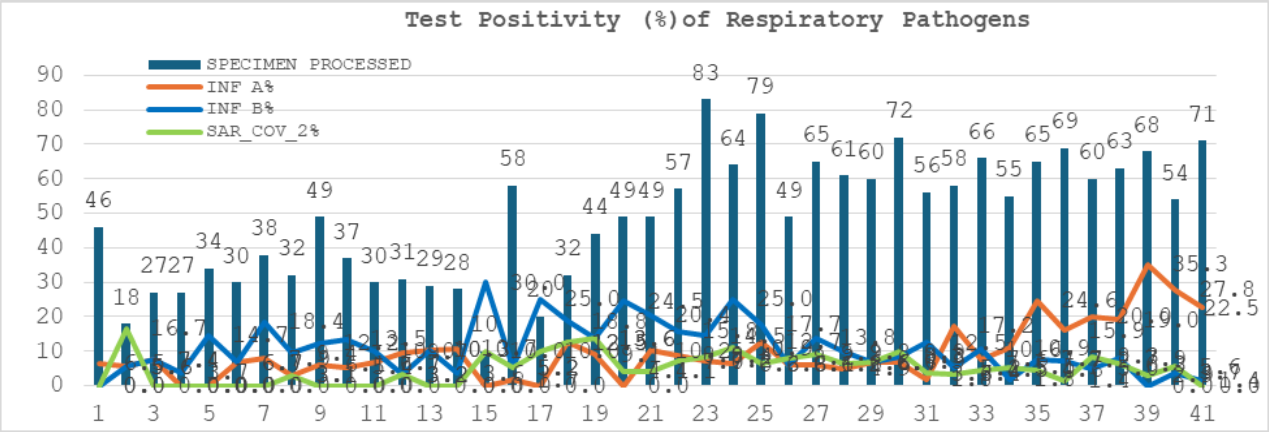
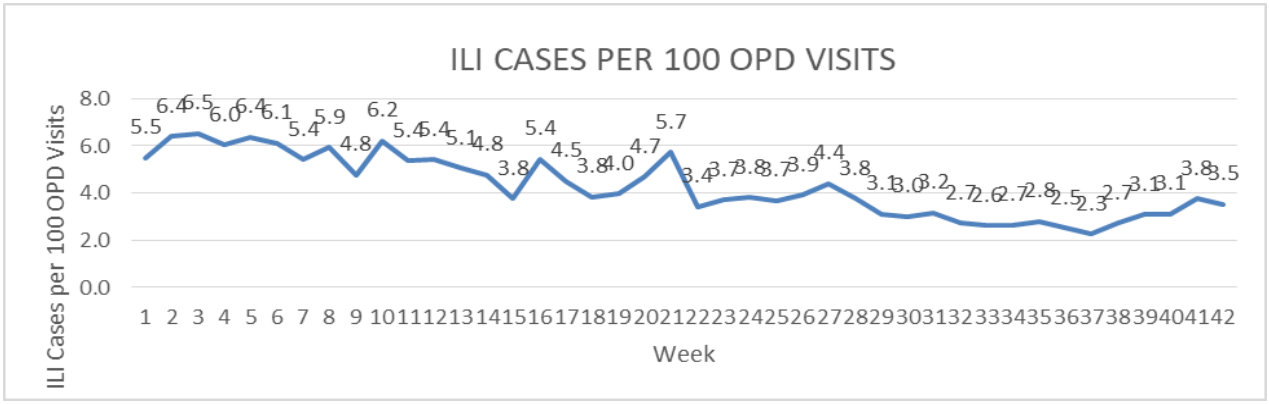
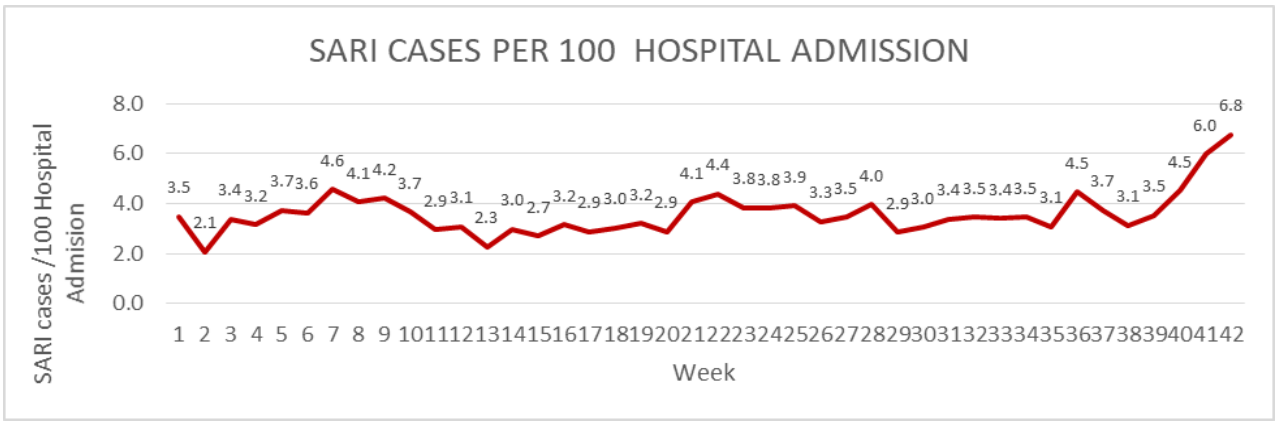


Figure 1 Test positivity of Respiratory Pathogens-2026





Continuous monitoring of ILI, SARI, laboratory positivity and circulating virus types is important for identifying changes in influenza activity and informing public health decisions.

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4. World Health Organization. How can I avoid getting the flu? Geneva: WHO; 2026. Available from: https://www.who.int/news-room/questions-and-answers/item/how-can-I-avoid-getting-the-flu

To be Continued….

Table 1: Distribution of Notified Diseases reported by Medical Officers of Health 07th – 13th Sep 2026 (37th Week)

RDHS	Dengue Fever		Dysentery		Encephalitis		En. Fever		F. Poison-		Leptospirosis		Typhus		Viral Hep.		H. Rabies		Chickenpox		Meningitis		Leishman.		Tuberculosis		Leprosy		WRCD	
	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	T*	C**
Colombo	11	19327	0	18	0	2	0	4	0	21	1	237	0	2	0	17	0	0	2	411	2	85	1	5	51	1368	2	119	28	97
Gampaha	15	16063	2	35	0	34	0	0	0	25	1	373	0	6	2	12	0	0	7	605	3	225	15	54	31	762	0	56	0	74
Kalutara	31	7773	0	35	0	4	0	11	0	63	6	321	9	18	6	24	0	0	6	656	3	106	2	15	5	406	1	74	62	90
Kandy	15	6574	1	41	0	9	0	7	0	38	0	146	0	27	0	19	0	0	8	503	0	101	0	43	18	379	0	19	0	100
Matale	139	2351	0	20	0	5	0	2	0	7	4	160	0	3	1	9	0	0	10	170	2	68	0	381	3	137	2	27	5	91
Nuwara Eliya	54	1116	3	40	1	6	0	3	6	22	9	227	2	42	2	16	0	0	11	369	7	159	0	2	3	188	0	9	13	100
Galle	56	6189	0	30	0	13	0	8	0	60	3	419	0	33	0	25	0	0	2	773	0	135	0	2	10	319	0	48	30	89
Hambantota	26	2812	1	48	0	2	2	2	1	47	4	208	0	28	1	14	0	0	2	175	1	55	11	401	1	105	0	22	92	100
Matara	25	6471	7	29	0	4	1	3	1	12	1	297	2	20	0	17	0	0	0	404	1	142	0	108	11	119	0	26	0	100
Jaffna	12	1079	0	76	0	6	0	20	0	96	1	56	0	243	1	26	0	0	3	291	0	44	0	1	4	134	0	12	87	100
Kilinochchi	101	748	1	13	1	2	0	8	0	1	10	86	0	12	0	3	0	1	5	102	4	18	0	2	1	27	3	5	0	100
Mannar	12	549	0	1	0	3	0	0	0	3	0	47	0	4	0	3	0	0	0	65	0	21	0	5	0	39	0	5	17	100
Vavuniya	33	371	0	22	0	4	0	2	0	8	8	110	0	4	0	1	0	0	14	137	1	35	1	24	0	52	0	7	0	83
Mullaitivu	7	112	0	6	0	2	0	1	0	13	2	37	0	6	0	2	0	0	0	13	0	10	0	10	2	26	0	7	0	100
Batticaloa	15	1579	0	67	0	14	0	2	0	21	3	149	1	3	0	16	0	0	5	262	1	78	2	24	3	117	0	84	0	100
Ampara	0	674	0	53	0	2	0	1	0	16	0	129	0	2	0	17	0	0	1	279	0	64	0	17	0	42	1	22	7	100
Trincomalee	5	764	0	25	0	6	0	2	0	15	0	79	0	8	0	4	0	0	1	189	1	32	4	22	0	108	0	7	100	100
Kurunegala	17	3446	0	23	0	18	0	3	0	66	3	320	0	31	0	13	0	1	4	614	1	298	1	330	7	235	2	53	11	97
Puttalam	4	1885	0	19	0	11	0	0	0	9	1	198	0	26	1	8	0	4	0	131	0	141	4	19	10	141	0	24	12	93
Anuradhapura	0	1132	0	19	0	9	0	3	0	53	0	205	0	26	0	13	0	0	0	448	0	125	0	576	5	194	0	50	0	100
Polonnaruwa	1	833	0	25	0	7	0	2	0	59	1	226	0	7	1	26	0	1	2	385	1	58	0	396	0	63	0	67	0	90
Badulla	3	1730	0	34	0	8	0	3	0	13	0	208	0	44	0	120	0	0	10	349	0	206	2	99	5	187	0	15	3	100
Monaragala	15	1290	0	15	0	3	0	2	1	11	1	227	0	31	0	56	0	1	0	191	1	87	0	173	2	96	0	19	27	80
Ratnapura	34	5363	0	31	0	8	0	7	1	21	18	726	1	32	0	15	0	0	3	293	2	91	0	161	2	295	0	31	3	100
Kegalle	4	2787	1	47	0	6	0	5	1	29	0	322	0	11	0	13	0	0	1	568	1	80	0	13	8	237	0	9	0	93
Kalmunai	0	1037	0	44	0	3	0	1	0	22	0	63	0	1	0	3	0	0	2	384	0	36	0	0	3	90	0	26	6	100
SRILANKA	**635**	**94055**	**16**	**816**	**2**	**191**	**3**	**102**	**11**	**751**	**77**	**5576**	**15**	**670**	**15**	**492**	**0**	**8**	**99**	**8767**	**32**	**2500**	**43**	**2883**	**185**	**5866**	**11**	**843**	**19**	**95**

Source: WRCD module of the EPINET.
A = Cases reported during the current week;
T*=Timeliness refers to returns received on or before 13th Sep, 2026.
B = Cumulative cases for the year.
Total number of reporting units 363
C**-Completeness;
C**-Completeness;

Table 2: Selected Vaccine Preventable Diseases & AFP 07th – 13th Sep 2026 (37th Week)

Disease	No. of Cases by Province									Number of cases during current week in 2026	Number of cases during same week in 2025	Total number of cases to date in 2026	Total number of cases to date in 2025	Difference between the number of cases to date in 2026 & 2025
	W	C	S	N	E	NW	NC	U	Sab					
AFP[1]	00	00	00	00	00	00	00	00	01	01	01	45	45	0%
Diphtheria	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Mumps[2]	00	00	00	00	01	00	01	00	00	02	03	106	109	-2.7%
Measles[3]	00	00	00	00	00	00	00	00	00	00	00	60	02	2900 %
Rubella[3]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
CRS[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Tetanus[2]	00	00	00	00	00	00	00	00	00	00	01	04	04	0 %
Neonatal Tetanus[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Japanese Encephalitis[3]	00	00	00	00	00	00	00	00	00	00	01	00	04	-100 %
Whooping Cough[2]	00	00	00	00	00	00	00	00	00	00	00	21	16	31.2 %

Key to Table 2

Provinces: **W**: Western, **C**: Central, **S**: Southern, **N**: North, **E**: East, **NC**: North Central, **NW**: North Western, **U**: Uva, **Sab**: Sabaragamuwa.

Data Sources:
Weekly Return of Communicable Diseases: Diphtheria, Mumps, Tetanus, Neonatal Tetanus, Whooping Cough.
Special Surveillance: AFP, Measles, Rubella, CRS.

AFP[1] = No Polio cases
Mumps[2], CRS[2], Tetanus[2], Neonatal Tetanus[2], Whooping Cough[2]—Clinically and/ or laboratory confirmed cases
Measles[3], Rubella[3], Japanese Encephalitis[3]— Laboratory Confirmed cases

AFP—Acute Flaccid Paralysis
CRS = Congenital Rubella Syndrome
NA = Not Available

AFP and all Vaccine Preventable Diseases should be investigated by the MOH Personally.

All patients presenting with fever and a maculopapular rash must be promptly notified and investigated. Blood samples and throat swabs should be collected at the appropriate time window

Comments and contributions for publication in the WER Sri Lanka are welcome. However, the editor reserves the right to accept or reject items for publication. All correspondence should be mailed to The Editor, WER Sri Lanka, Epidemiology Unit, P.O. Box 1567, Colombo or sent by E-mail to **chepid@sltnet.lk.** The Epidemiology Unit should be formally acknowledged in all resulting publications as the primary data source.

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